

Abemaciclib + endocrine therapy

Adjuvant oral treatment for high-risk HR-positive/HER2-negative early breast cancer

WHAT THIS REGIMEN IS FOR

Abemaciclib with endocrine therapy for selected adults with HR-positive, HER2-negative, node-positive early breast cancer at high risk of recurrence.

MEDICINES

Abemaciclib

Endocrine therapy

TREATMENT JOURNEY

Every day	Alongside it	First 2 months	Next 2 months	Treatment duration
Abemaciclib 150 mg twice daily, with or without food, at approximately the same times each day unless your dose has been changed.	Continue the endocrine treatment prescribed for you.	Blood count, kidney/liver and bone profile are checked every 2 weeks.	These tests are usually monthly, then as clinically indicated.	Abemaciclib is taken continuously for 2 years or until progression/unacceptable toxicity, according to the NCCP source and your treatment plan.

REGIMEN-SPECIFIC POINT

Because this is continuous oral treatment, early diarrhoea management, medication-interaction review and the first months of blood monitoring are central to safe treatment.

MONITORING / SUPPORT

- Start anti-diarrhoeal treatment at the first sign of loose stools exactly as advised by your oncology team.
- Swallow tablets whole; do not chew, split or crush.
- Avoid grapefruit/grapefruit juice. If a dose is missed or vomited, take the next dose at the scheduled time rather than an extra dose.

Where changes may show up

Patient-first anatomical guide · report the symptom rather than trying to diagnose the cause

Bowel

Diarrhoea is an important early toxicity. Start the advised treatment promptly and contact oncology if it persists or becomes severe.

Blood + infection

Neutropenia can occur. Fever, shivers or sudden illness needs immediate oncology contact.

Liver

Liver enzymes can rise. Yellow eyes, dark urine or severe itching should be reported.

Blood clots

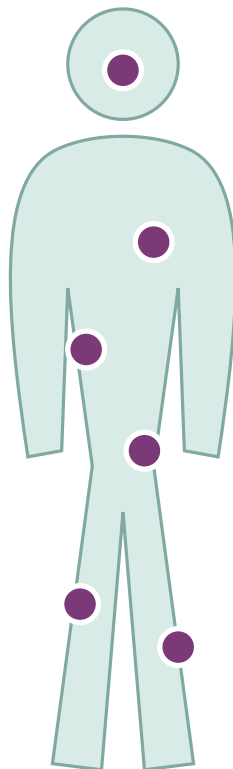
One-sided leg swelling/pain, sudden chest pain or breathlessness can indicate a clot and needs urgent assessment.

Lungs

New or worsening cough or breathlessness needs review; rare interstitial lung disease/pneumonitis is recognised.

Fatigue

Tiredness can occur. A marked change can also reflect anaemia, dehydration, infection or another cause.



WHEN TO CALL

Call oncology urgently for fever, persistent or severe diarrhoea, dehydration, new breathlessness, one-sided leg swelling, jaundice, significant bleeding or marked weakness. Severe chest pain or breathing difficulty: call 112/999.



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